CHIMALAYA NEPAL

IMPROVING MATERNAL AND CHILD HEALTH

Every newborn, a home visit

Impact of Chimalaya's postnatal home-visit programme, 2017–2026

Chimalaya Charity Nepal
27 October 2026



A newborn and family with their Bag4Life bag, which every family receives at the first home visit.

At a glance

Chimalaya Charity is a Danish–Nepalese NGO working to improve maternal and child health. Since 2017 its home-visit nurses, based at the mothers' group clinic in Bode, Madhyapur Thimi, have visited every newborn they can reach in the surrounding community: a first visit in the baby's first weeks, two follow-up visits, and a final visit around six months of age. At each visit they examine mother and baby, support breastfeeding, and counsel the family on nutrition, hygiene and immunisation.

This report draws on the programme's own records, with every family visited from January 2017 to 08/2026.

1,815
newborns visited at home since 2017

11 days
median age at the first home visit

89%
received all three newborn visits

59%
exclusively breastfed for the full 6 months

Reach

Who we visit

About 189 newborns a year from 42 localities around Bode and Thimi. 46% of them girls.

Continuity

We stay with families

Since 2023, about 99% of families have received every planned follow-up visit, including the visit at six months.

Breastfeeding

The core of every visit

96% of babies are exclusively breastfed at the third visit, around 11 weeks of age, and 59% for the full six months.

Growth

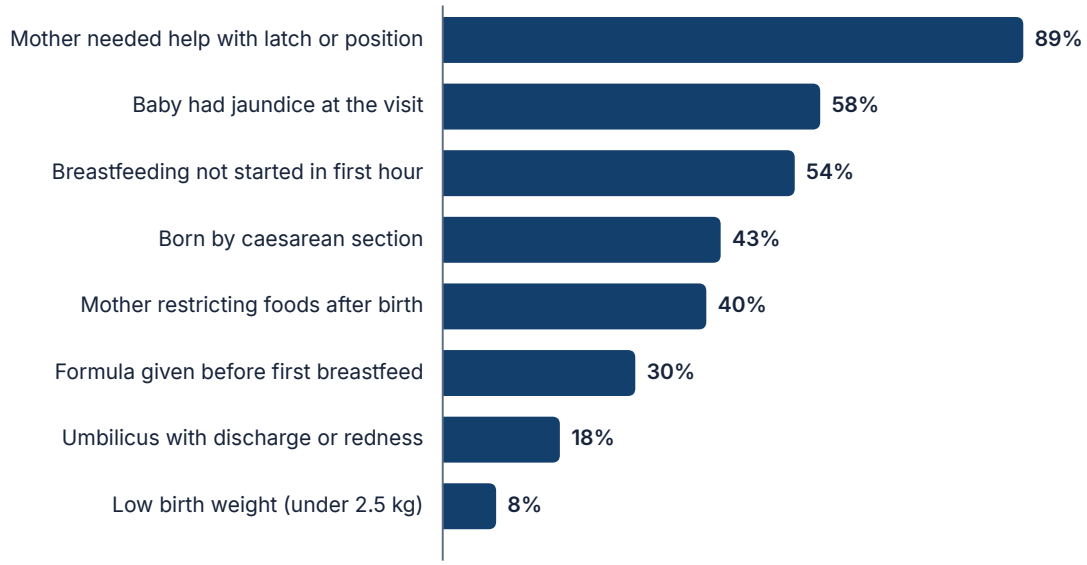
Measured at six months

Only 2% of babies were underweight against the WHO growth standards at around seven months of age.

The need

Almost every baby in our area is now born in a health facility: 96% in our 2017–2019 records. But mother and baby usually go home within a day or two, and the weeks that follow are when breastfeeding is established, when newborn problems such as jaundice or an infected umbilical cord appear, and when families have the least professional support. What our nurses found at the first visit shows why that support matters.

What home visitors found at the first visit



Share of 584 babies with a detailed first-visit record, 2017–2019 (the period for which the full clinical checklist is available in digital form). Latch and position: at least one of the eight positioning and attachment checks on the breastfeeding observation checklist was not met.

Nine in ten mothers needed hands-on breastfeeding help

At the first visit, 89% of mothers had at least one problem with how the baby was positioned or attached at the breast, and only 46% had started breastfeeding within an hour of birth. After a caesarean section, 50% of babies were given formula before their first breastfeed, compared with 13% after a normal birth.

What we do



A head-to-toe newborn examination during a home visit.



Handing over the Bag4Life bag at the first visit.

Every newborn in the clinic's area is offered the same sequence of visits. The timings below are what actually happened, measured from the programme's records.

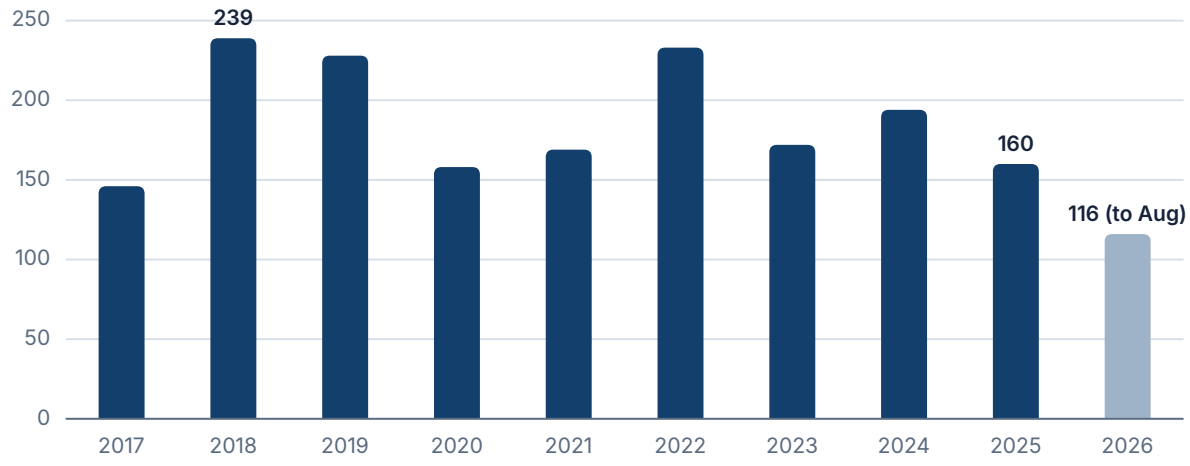
Visit	Typical timing	What happens
1st	baby 11 days old	Newborn and mother examination, screening for danger signs, breastfeeding position and attachment, hygiene advice. The family receives a Bag4Life bag.
2nd	18 days later	Weight check, feeding problems, reinforcing exclusive breastfeeding.
3rd	38 days after that	Exclusive breastfeeding, illness since the last visit, mother's nutrition, immunisation and family planning.
6 months	baby about 7 months old	Growth measurement, introducing complementary foods, invitation to the mother groups.

94% of families received the Bag4Life bag: a towel, changing mat, reusable nappies, a digital thermometer, cotton wool, and Nepali-made soap and baby oil.



Reach

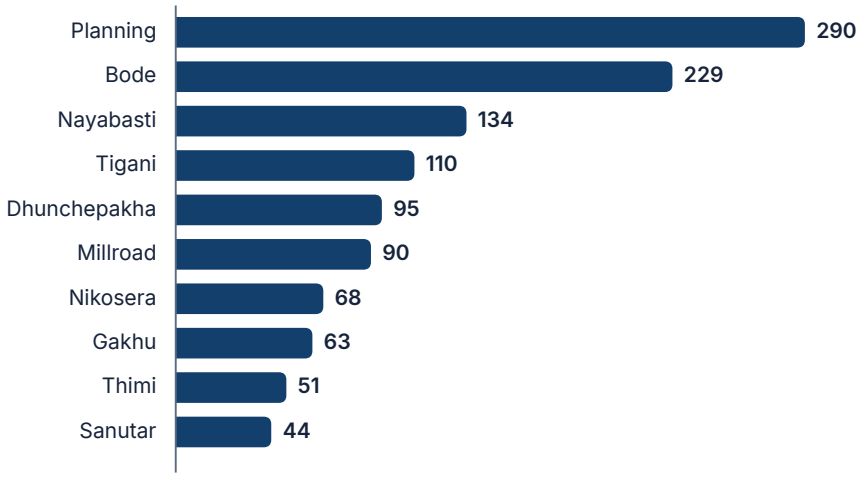
Newborns visited at home, by year



Babies by year of their first home visit. 2026 covers January to 08/2026. Outreach-camp babies, recorded separately, are not included.

Since January 2017 the programme has visited 1,815 newborns: on average 189 a year, or about 16 a month in recent years. Families come from 42 named localities around the clinic, from Planning and Bode to Nayabasti, Tigani and Dhunchepakha.

Where the families live



Ten localities with the most babies visited, 2017–2026. 641 further babies live in smaller localities or were recorded with a landmark only.

Continuity of care

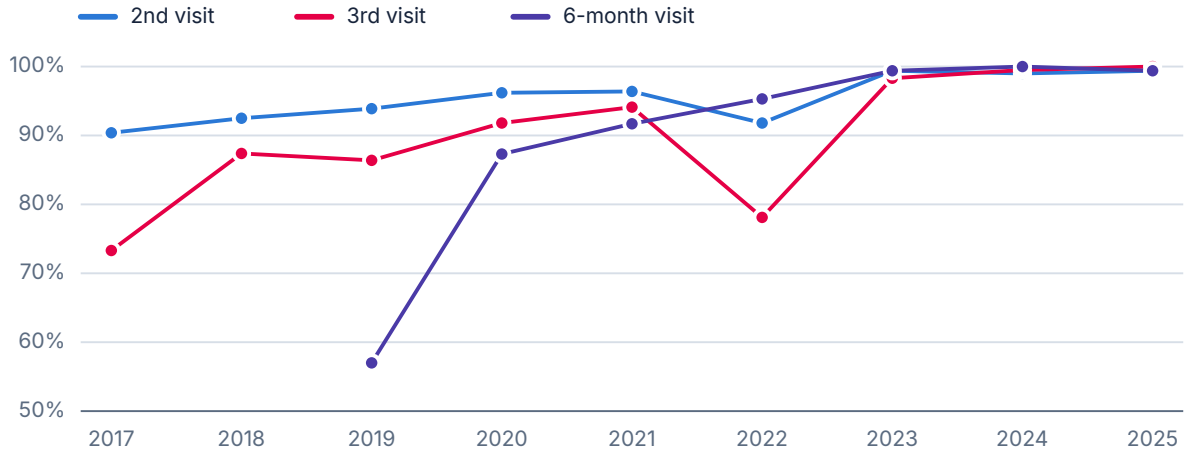
A single visit can solve an immediate problem; a series of visits builds the habits that last. The register shows that families stay with the programme.

96% received the 2nd visit

90% received the 3rd visit

89% received the 6-month visit

Share of babies who received each follow-up visit, by year of first visit



Only babies old enough for the visit are counted. The six-month visit is measured from 2019, when the six-month survey started. Part of the shortfall in earlier years is missing paperwork rather than missed visits: of the visits without a date, nearly all have no reason recorded, and only 2 families are recorded as declining.

Follow-up has become close to universal: for babies first visited in 2023 to 2025, around 99% of families received every follow-up visit. Visits now take place at the intended ages. The second visit comes a median of 18 days after the first, and the six-month visit at 7 months of age.

Outcomes at three and six months

Since 2021 the third visit and the six-month visit have been recorded in digital surveys, covering 978 and 965 babies so far. They show how the families we visit are feeding and growing their babies during and at the end of the newborn programme.

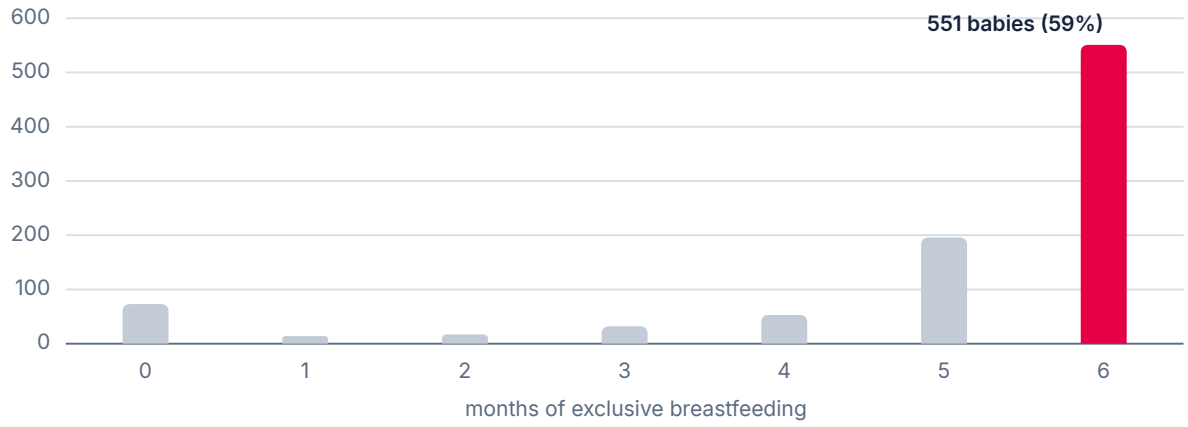
96%
exclusively breastfed at the third visit, around 11 weeks of age

59%
of the same babies exclusively breastfed for the full six months

99%
of mothers breastfeeding with good attachment and position at the third visit

At the third visit nearly every baby is still exclusively breastfed. Following the same 860 babies to the six-month visit, 59% kept it up until six months. The drop happens after the third visit, in the four months when no visit is scheduled.

How long babies were exclusively breastfed



Months of exclusive breastfeeding reported by the mother at the six-month visit, 2021–2026, 936 babies.

WHO recommends exclusive breastfeeding, with no water, other milk or food, for the full six months. Where other food came earlier, mothers most often said they did not have enough breast milk (46% of reasons given). Our counselling on breastfeeding technique and milk supply targets exactly this.

Healthy growth

At a median age of 7 months, 2% of babies were underweight and 9% were short for their age, against the WHO Child Growth Standards. Half weighed at or above the WHO median.

Advice that reaches every family

At the six-month visit, 964 of 965 families were counselled on nutrition for mother and child, complementary food, immunisation and family planning.

93% of the mothers seen at six months confirmed that the Chimalaya team had visited them at home in the newborn period.

Looking ahead



A mother group at the clinic in Bode.

The home-visit programme now reaches almost every family it enrols, through all four visits. With continued support, the next steps are:

- **Keep every family in the programme** as enrolment grows, so that the 99% follow-up achieved since 2023 holds.
- **Start breastfeeding support earlier**, before and right after birth, because half of babies were not breastfed within the first hour and nine in ten mothers needed help with latch and position.
- **Carry families on into the mother groups**, which run from six months to two years. 95% of families are invited at the six-month visit; recording who joins would show how far the programme reaches beyond the first six months.
- **Add a contact between the third visit and six months.** 96% of babies are exclusively breastfed at the third visit but only 59% reach six months, most often because mothers feel they do not have enough milk, or go back to work. A visit or call at four months is when that support would count.
- **Digitise the first and second visits**, as the third and six-month visits already are, so that outcomes can be followed for every baby from birth.

To be completed by Chimalaya
Programme cost per family (four visits and a Bag4Life bag), the funding needed for the coming year, and how to contribute.

About the data

This report uses four sets of records kept by Chimalaya's home-visit team:

- **Home-visit register** (2017–2026): one row per baby with the date of each visit. It is the basis for reach, timing and continuity.
- **Detailed first-visit checklist** (2017–2019, 584 babies): birth history, examination of mother and baby, and breastfeeding observation.
- **Third-visit survey** (2021–2026, 978 babies): breastfeeding, illness and growth at around 11 weeks, collected digitally.
- **Six-month survey** (2021–2026, 965 babies): feeding, growth and counselling, collected digitally.

The records were cleaned and anonymised before analysis. Names, phone numbers and addresses were removed. Typing errors in dates and units were corrected using rules that are logged for every record, and dates that could not be resolved were left out rather than guessed. Shares are always calculated over the babies for whom the item was recorded, and visits are only counted as missed once the baby was old enough to have received them.

Some limits to keep in mind:

- The detailed first-visit data covers 2017–2019 only.
- The answer codes in the 2017–2019 checklist were checked against the paper forms of 2017 and 2018; two codes that the spreadsheet numbers differently were inferred from the data.
- Growth is compared with WHO reference values by whole month of age, which is an approximation.
- The figures describe the families in the programme. Without a comparison group they show what was achieved, not how much is due to the visits alone.

The full method, the data-quality report and the code are available from Chimalaya on request.

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